G1 CLIMBING + FITNESS · PERSONAL TRAINING

# First-Session Assessment

Trainer use. A NASM-informed starting point for programming, not a medical exam or diagnosis.

**DRAFT V0.1**
Pending G1 management approval

Client | Date | Session length (min)

Trainer

**Scope.** Personal trainers observe movement to plan exercise. We don't diagnose, treat or rehabilitate injuries. Describe what you see in plain words (for example "left shoulder sits higher"), and refer out when something is outside our scope. Bodies differ: adapt or skip any screen that doesn't suit the client, and treat differences as information, not faults. Keep nutrition advice general, and suggest a qualified professional for medical or eating concerns.

## 1 · BEFORE WE START

- ☐ Intake reviewed with the client
- ☐ Waiver and policies on file (per G1 process)
- ☐ Client knows they can skip or stop anything
- ☐ Any "prefer to discuss" answers talked through privately
- ☐ Goals confirmed in the client's own words
- ☐ Explained what today is, and isn't
- ☐ Hands-on adjustment preference checked

**PLACEHOLDER:** G1 process for waiver on file, incident reporting, emergency action plan (first aid, AED) and medical-clearance requirements. Pending G1 management approval.

## 2 · TODAY'S CHECK-IN

Energy today (1 low, 5 high) | Sleep last night (hrs) | Pain right now (0 none, 10 worst) | Where?

Anything changed since the intake form?

Plan for today (decision criteria: pending G1 management approval)

◯ Proceed as planned ◯ Proceed with modifications ◯ Pause and suggest a healthcare check-in

**Stop and refer.** If a client has chest pain or pressure, faints, or has severe dizziness or trouble breathing: stop, stay with them and follow G1's emergency action plan **[placeholder: pending G1 approval]**. For anything else here (unusual shortness of breath, sudden sharp or radiating pain, new numbness or tingling, a joint that feels unstable after an injury, or pain that stops a test): pause, describe what you saw, and suggest they speak with a licensed healthcare professional. Don't diagnose or guess. Note what happened per G1's incident process **[placeholder: pending G1 approval]**.

## 3 · RESTING MEASURES (optional; record only what helps the client's goals)

Resting heart rate (bpm) | Blood pressure (if G1 policy allows) | Height | Weight (only if the client wants it)

**PLACEHOLDER:** G1 policy on resting heart rate, blood pressure and body measurements: who may take them, any thresholds, and how results are recorded.

## 4 · POSTURE SNAPSHOT (optional, standing; describe, don't label)

| CHECKPOINT | WHAT I SEE (FRONT, SIDE, BACK) |
|---|---|
| Feet and ankles | |
| Knees | |
| Hips and low back | |
| Shoulders | |
| Head and neck | |

Client _____

**Observe, don't diagnose.** These screens show how the client moved today, in these shoes, with these instructions. Use them to choose a starting point and to re-check later. Stop any test that causes pain.

**A · Overhead squat** 5 slow reps, feet shoulder-width, toes forward, arms overhead. Watch from the front, side and back.

| | |
|---|---|
| Front: feet and knees | ☐ Feet turn out ☐ Knees move inward ☐ Knees move outward |
| Side: heels, hips and low back | ☐ Heels rise ☐ Excessive forward lean ☐ Low back arches ☐ Low back rounds or tucks under |
| Side: shoulders | ☐ Arms fall forward |
| Back: feet and hips | ☐ Feet flatten ☐ Weight shifts to one side |
| Retests (optional) | ☐ Heels raised: pattern improved ☐ Hands on hips: low-back arch improved |
| Overall | ○ Smooth ○ Small differences ○ Clear differences ○ Stopped (pain or limit) |
| Depth | ○ Thighs about parallel ○ Limited |
| Pain during the squat | ○ None ○ Yes, where: _____ |

**B · Single-leg squat** Hands on hips, 5 reps each side. NASM names knee inward and forward lean; the rest are common extras.

| OBSERVATION | LEFT | RIGHT |
|---|---|---|
| Knee moves inward | ☐ | ☐ |
| Knee moves outward | ☐ | ☐ |
| Hip hikes or drops | ☐ | ☐ |
| Trunk leans forward or low back arches | ☐ | ☐ |
| Loses balance | ☐ | ☐ |
| Pain or discomfort (note where below) | ☐ | ☐ |

**C · Pushing and pulling** A push-up or standing band/cable press, then a row or band pull. Watch from the side.

| | |
|---|---|
| Push | ☐ Low back arches ☐ Shoulders rise toward ears ☐ Head moves forward |
| Pull | ☐ Low back arches ☐ Shoulders rise toward ears ☐ Head moves forward |

**D · Climbing add-ons** Optional G1 additions, not NASM protocols. Skip any that cause pain. No max-effort finger or hangboard testing in session one.

| SIDE-BY-SIDE CHECKS | LEFT | RIGHT |
|---|---|---|
| Knee-to-wall distance, heel down (cm) | _____ | _____ |
| Seated trunk rotation limited? | ☐ | ☐ |

| | |
|---|---|
| Overhead reach (back to wall) | ☐ Reaches up without arching ☐ Ribs flare or low back arches ☐ Reach is limited |
| Supported hang (feet on floor, 10 s or less): shoulder position only | ☐ Shoulders stay set ☐ Shoulders shrug to ears ☐ Skipped (finger, wrist or elbow symptoms) |

What stood out, and what the client said they felt (including any pain or discomfort, with where)

Client

## 5 · BASELINE SNAPSHOT (optional; pick what fits; quality before quantity; stop on pain)

| TEST | VERSION / SETUP | RESULT | NOTES (FORM, EFFORT 0-10) |
|---|---|---|---|
| Plank hold | | | |
| Push-up (wall, incline, knees, full) | | | |
| Sit-to-stand, 30 seconds | | | |
| Easy cardio, 5 minutes | | | |
| Climbing movement (optional) | | | |

## 6 · WHAT I NOTICED, AND THE TRAINING FOCUS (hypotheses to test through programming, not diagnoses)

| WHAT I NOTICED | STARTING EMPHASIS IDEA | PRIORITY |
|---|---|---|
| | | |
| | | |
| | | |
| | | |

| QUICK REFERENCE: WHERE PROGRAMMING OFTEN STARTS (starting ideas, not rules; not a read on which muscles are tight or weak; check against G1's training standards) | |
|---|---|
| **Feet turn out or flatten; heels rise** | Calf and ankle mobility; foot and lower-leg activation; stance cues; a temporary heel lift while the ankle improves |
| **Knees move inward** | Adductor and outer-thigh length; glute activation; foot and ankle mobility; knee-tracking cues |
| **Excessive forward lean** | Calf and hip-flexor length; shin, glute and back-extensor activation |
| **Low back arches** | Hip-flexor and lat length; glute and deep-core activation; rib and pelvis cues. Hands-on-hips retest: lats or core? |
| **Low back rounds or tucks under** | Hamstring and hip mobility; glute and back-extensor activation; hip-hinge practice |
| **Arms fall forward** | Lat and chest length; mid-back and rotator-cuff activation; thoracic mobility |
| **Shoulders rise or head moves forward (push, pull, hang)** | Upper-trap and neck length; scapular control work; deep neck-flexor activation |

## 7 · STARTING PLAN

Starting OPT phase (most new clients start in Phase 1)

◯ Phase 1 Stabilization Endurance ◯ Phase 2 Strength Endurance ◯ Other: _____

Why this phase? Sessions per week Next session

Homework (1-2 simple items) First progress review

Client's own words after today: what felt good, what worried them

Referral or follow-up (a suggestion only; document what you said, and never record "cleared"; invite the client to share any guidance in their own words)

◯ None needed ◯ Suggested a healthcare check-in about: _____

OPT phases: 1 Stabilization Endurance · 2 Strength Endurance · 3 Muscular Development · 4 Maximal Strength · 5 Power. This sheet is NASM-informed. It is not affiliated with or endorsed by NASM, and NASM and OPT are trademarks of the National Academy of Sports Medicine. Sources: SOURCES.md.